CMF · oral cyclophosphamide

National Cancer Control Programme (NCCP) 00377 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

Cyclophosphamide tablets on Days 1–14, with intravenous (IV) methotrexate and fluorouracil on Days 1 and 8 of each 28-day cycle. Six cycles for adjuvant treatment; metastatic treatment continues while beneficial and tolerated.

STEP 1

Before the first visit

Blood tests, medicines review and the consent discussion come first. Confirm dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing before fluorouracil and review methotrexate kidney clearance.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.

STEP 4

Home and next review

Take the prescribed cyclophosphamide tablets only on Days 1–14. Keep them safely and follow the pharmacy handling advice. Keep the Day 8 IV appointment and confirm the next cycle before restarting tablets.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

Allow time for blood review, preparation, treatment and observation. Your unit confirms the full visit duration. Arrange transport or an escort where advised.

Phase-specific visit pattern

Cyclophosphamide tablets on Days 1–14, with IV methotrexate and fluorouracil on Days 1 and 8 of each 28-day cycle. Six cycles for adjuvant treatment; metastatic treatment continues while beneficial and tolerated. No take-home infusion pump is part of this schedule. Port de-accessing after a visit is different from removal of the implanted port.

Sources: NCCP 00377 v6 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.